

Cochlear Implants

Policy Number: CS019.S

Effective Date: September 1, 2025

[➔ Instructions for Use](#)

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Related Community Plan Policies

- [Durable Medical Equipment, Orthotics, Medical Supplies, and Repairs/Replacements](#)
- [Hearing Aids and Devices Including Wearable, Bone-Anchored, and Semi-Implantable](#)

Commercial Policy

- [Cochlear Implants](#)

Application

This Medical Policy does not apply to the states listed below; refer to the state-specific policy/guideline, if noted:

State	Policy/Guideline
Idaho	None
Indiana	None
Kansas	Cochlear Implants (for Kansas Only)
Kentucky	Cochlear Implants (for Kentucky Only)
Nebraska	Cochlear Implants (for Nebraska Only)
New Jersey	Cochlear Implants (for New Jersey Only)
New Mexico	Cochlear Implants (for New Mexico Only)
North Carolina	Cochlear Implants (for North Carolina Only)
Ohio	Cochlear Implants (for Ohio Only)
Pennsylvania	Cochlear Implants (for Pennsylvania Only)
Tennessee	Cochlear Implants (for Tennessee Only)

Coverage Rationale

[➔ See \[Benefit Considerations\]\(#\)](#)

Non-hybrid cochlear implantation is proven and medically necessary under certain circumstances for bilateral sensorineural and/or for single sided or asymmetric [Sensorineural Hearing Loss](#) in adults ages 18 and older. For medical necessity clinical coverage criteria, refer to the InterQual® CP: Procedures, Cochlear Implantation.

[Click here to view the InterQual® criteria.](#)

Non-hybrid cochlear implantation is proven and medically necessary under certain circumstances for bilateral Sensorineural Hearing Loss in children ages 6 months or older and for single-sided or asymmetric Sensorineural Hearing Loss in children ages 9 months or older. For medical necessity clinical coverage criteria, refer to the InterQual® CP: Procedures, Cochlear Implantation (Pediatric).

[Click here to view the InterQual® criteria.](#)

Non-hybrid cochlear implantation for bilateral Sensorineural Hearing Loss in children younger than 6 months and for single-sided or asymmetric Sensorineural Hearing Loss in children younger than 9 months is experimental or investigational, due to lack of Food and Drug Administration (FDA) approval.

Hybrid cochlear implantation is proven and medically necessary under certain circumstances for Sensorineural Hearing Loss in adults ages 18 and older. For medical necessity clinical coverage criteria, refer to the InterQual® CP: Procedures, Cochlear Implantation.

[Click here to view the InterQual® criteria.](#)

Medical Records Documentation Used for Reviews

Benefit coverage for health services is determined by federal, state, or contractual requirements, and applicable laws that may require coverage for a specific service. Medical records documentation may be required to assess whether the member meets the clinical criteria for coverage but does not guarantee coverage of the service requested; refer to the guidelines titled [Medical Records Documentation Used for Reviews](#).

Definitions

Sensorineural Hearing Loss (SNHL): Occurs when there is damage to the inner ear (cochlea), or to the nerve pathways from the inner ear to the brain. Most of the time, SNHL cannot be medically or surgically corrected. This is the most common type of permanent hearing loss. [American Speech-Language-Hearing Association (ASHA)]

Applicable Codes

The following list(s) of procedure and/or diagnosis codes is provided for reference purposes only and may not be all inclusive. Listing of a code in this policy does not imply that the service described by the code is a covered or non-covered health service. Benefit coverage for health services is determined by federal, state, or contractual requirements and applicable laws that may require coverage for a specific service. The inclusion of a code does not imply any right to reimbursement or guarantee claim payment. Other Policies and Guidelines may apply.

CPT Code	Description
69930	Cochlear device implantation, with or without mastoidectomy

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HCPCS Code	Description
L8614	Cochlear device, includes all internal and external components
L8615	Headset/headpiece for use with cochlear implant device, replacement
L8616	Microphone for use with cochlear implant device, replacement
L8617	Transmitting coil for use with cochlear implant device, replacement
L8618	Transmitter cable for use with cochlear implant device or auditory osseointegrated device, replacement
L8619	Cochlear implant external speech processor and controller, integrated system, replacement
L8627	Cochlear implant, external speech processor, component, replacement
L8628	Cochlear implant, external controller component, replacement
V5273	Assistive listening device, for use with cochlear implant

Benefit Considerations

Cochlear implants external components (i.e., speech processor, microphone, and transmitter coil) are considered under the DME benefit, and the implantable components are considered under the medical-surgical benefit. Check the federal, state, or contractual requirements for benefit plan coverage. Refer to the Medical Policy titled [Durable Medical Equipment, Orthotics, Medical Supplies, and Repairs/Replacements](#).

U.S. Food and Drug Administration (FDA)

This section is to be used for informational purposes only. FDA approval alone is not a basis for coverage.

For information on non-hybrid cochlear implants, refer to the following FDA website for Premarket Approvals (use product code MCM): <https://www.accessdata.fda.gov/scripts/cdrh/cfdocs/cfPMA/pma.cfm>. (Accessed February 28, 2025)

For information on hybrid cochlear implants, refer to the following FDA website for Premarket Approvals (use product code PGQ): <http://www.accessdata.fda.gov/scripts/cdrh/cfdocs/cfPMN/pmnm.cfm>. (Accessed February 28, 2025)

References

American Speech-Language-Hearing Association (ASHA). Sensorineural Hearing Loss. Available at: <https://www.asha.org/public/hearing/sensorineural-hearing-loss/>. Accessed February 28, 2025.

Policy History/Revision Information

Date	Summary of Changes
04/01/2026	Template Update Removed content/language pertaining to the state of Louisiana
10/01/2025	Application Nebraska Added language to indicate this Medical Policy does not apply to the state of Nebraska; refer to the state-specific policy version
09/01/2025	Medical Records Documentation Used for Reviews - Added language to indicate: - Benefit coverage for health services is determined by the federal, state, or contractual requirements, and applicable laws that may require coverage for a specific service - Medical records documentation may be required to assess whether the member meets the clinical criteria for coverage but does not guarantee coverage of the service requested; refer to the guidelines titled Medical Records Documentation Used for Reviews Supporting Information - Archived previous policy version CS019.R

Instructions for Use

This Medical Policy provides assistance in interpreting UnitedHealthcare standard benefit plans. When deciding coverage, the federal, state or contractual requirements for benefit plan coverage must be referenced as the terms of the federal, state or contractual requirements for benefit plan coverage may differ from the standard benefit plan. In the event of a conflict, the federal, state or contractual requirements for benefit plan coverage govern. Before using this policy, check the federal, state or contractual requirements for benefit plan coverage. UnitedHealthcare reserves the right to modify its Policies and Guidelines as necessary. This Medical Policy is provided for informational purposes. It does not constitute medical advice.

UnitedHealthcare may also use tools developed by third parties, such as the InterQual® criteria, to assist us in administering health benefits. The UnitedHealthcare Medical Policies are intended to be used in connection with the independent professional medical judgment of a qualified health care provider and do not constitute the practice of medicine or medical advice.